



Problem Statement & Proposed Solution

Why emergency patients lose the golden hour, and how Medreach fixes the information gap between citizens and hospitals.

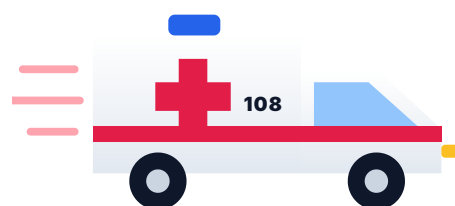
☆ MPOne Idea & Innovation Hackathon 2026 🏠 Challenge 5 · AI for Public Services · Healthcare

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1 The problem, in one real-life scene

11 pm in Bhopal. A 62-year-old man has chest pain and is sweating. His son drives him to the nearest hospital, where there is no cardiologist on duty and no free ICU bed. They are sent across the city. Precious minutes are gone.

The family did nothing wrong. They simply had no way to know which hospital could treat this emergency at this moment, and no hospital knew they were coming.



The scale of the problem. India recorded 1,68,491 road-accident deaths in 2022 (Ministry of Road Transport & Highways, "Road Accidents in India 2022"). Heart attacks, strokes, burns and obstetric emergencies add many more time-critical cases. For all of them, the first hour decides the outcome.

2 Root causes we identified



Nearest ≠ capable

Maps show the closest hospital, not whether it has the specialist, the ICU bed or a working CT scanner right now.



No live, trusted status

Bed and doctor availability is not published, or is outdated with no time stamp or owner.



No acceptance before travel

Families arrive unannounced. Hospitals cannot prepare, and refusals happen at the door.



Language & panic

In panic, people cannot fill forms. Many prefer Hindi or Hinglish and speak rather than type.



People living alone

An elderly person alone cannot call around hospitals, arrange transport and inform family all at once.



Lost information

Allergies, ABHA / Ayushman details and history do not reach the ER doctor before the patient does.

3 Who is affected

Stakeholder	Today's pain
Patients & families	Wasted trips, refusals at the gate, delay in definitive care, high anxiety.
Hospitals (ER teams)	Unannounced critical arrivals, overcrowding, no time to prepare a bay or call a specialist.
Health department	No live picture of emergency capacity across hospitals; no data on referral delays.

4 Proposed solution: Medreach

Medreach is an emergency *coordination* platform, not a diagnosis app. It gets the patient to a hospital that can treat them and that has accepted them, before they leave home. It implements our flowchart step by step:



Root cause	How Medreach solves it
Nearest ≠ capable	AI turns the description into required capabilities (e.g. 24×7 emergency + cardiologist + ICU). A deterministic engine keeps only hospitals where the service exists, the specialist is on duty and the equipment works now, then ranks by time to care.
No live, trusted status	Authorised hospital staff (Hospital ID + Staff ID + OTP) update beds, ER status, roster and equipment. Every figure shows who / when / source; freshness is enforced (green / amber / red). Changes reach citizens in under a second.
No acceptance before travel	The hospital receives a referral with an AI SBAR pre-arrival note, accepts and assigns a receiving bay. Decline or no answer in 60 s → next hospital automatically.
Language & panic	Speak in Hindi / English / Hinglish, quick-pick chips, one follow-up question at a time, voice read-back of key steps.
People living alone	"I'm alone" mode: one tap shares location, alerts a trusted contact with a tracking link, sends the hospital request and requests an ambulance.
Lost information	OCR reads the ABHA / Ayushman card on the phone; details go to the hospital only with consent and with IDs masked.

5 What the citizen sees

AI understands: "may indicate heart attack", needs cardiologist + ICU

Only hospitals that can treat this now

Accepted by a named officer, bay Resus-02

"I'm alone" mode runs every step

Result: one decision, made with verified information, instead of a trial-and-error journey between hospitals. The hospital is ready when the patient arrives.